Submitted via email to *https://www.regulations.gov*

June 16, 2025

Dr. Mehmet Oz
Administrator for the Centers for Medicare & Medicaid Services

Thomas Keane, MD
Secretary for Technology Policy and National Coordinator for Health Information Technology

Centers for Medicare & Medicaid Services
Department of Health and Human Services
P.O. Box 8013
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Attention: CMS–0042–NC

RE: Request for Information on Health Technology Ecosystem [CMS–0042–NC]

Dear Dr. Oz and Dr. Keane:

The Pharmaceutical Care Management Association (PCMA) appreciates the opportunity to submit comments on the Request for Information issued by the Department of Health and Human Services (HHS) Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC) and published in the Federal Register on May 16, 2025 (RFI).[1]

PCMA is the national association representing America's pharmacy benefit managers (PBMs), which administer prescription drug plans and operate specialty pharmacies for more than 289 million Americans with health coverage through Fortune 500 companies, health insurers, labor unions, Medicare, Medicaid, the Federal Employees Health Benefits Program, and plans offered for sale on the Exchanges established by the Affordable Care Act. PBMs negotiate price concessions with manufacturers on their brand medications to improve the value of the Part D program. These price concessions are used to reduce premiums and improve the Part D benefit by providing access to preferred drugs with reduced cost sharing.

PCMA strongly supports efforts to reduce barriers to data access and exchange, and to use technology to accelerate the transition to value-based care (VBC) to improve health outcomes. We agree that while significant progress has been made in using health information technology (health IT) for these purposes, there are still opportunities and challenges to improve accessibility, interoperability, and exchange of health data to deliver better, safer, and more cost-effective care to patients.

[1] 90 Fed. Reg. 21034 (May 16, 2025).

Today PBMs conduct 94% of eligibility and benefit verifications electronically in real time.[2] In addition, the PBM industry has fully integrated and made available electronic pharmacy benefit tools such as electronic prior authorization (ePA), electronic prescribing (eRx), and Real Time Benefit Tools (RTBT) (also known as Real Time Prescription Benefit or RTPB) to pharmacies and providers. These tools offer many benefits to patients and providers, ranging from improved patient care and safety to lowering medication costs and reducing administrative burden, which in turn allows clinicians to spend more time on direct patient care.

Below we discuss ways in which the transition to greater use of these tools can be fostered.

1. Electronic Prior Authorization

Prior authorization (PA) is an essential tool used to ensure that outpatient drug benefits are administered as designed and that plan members receive the medication therapy that is safe, is effective for their condition, and provides the greatest value. Seventy-five percent (75%) of pharmacy prior authorizations are fully electronic and use the National Council for Prescription Drug Programs (NCPDP) SCRIPT electronic standard for PA.[3]

All PBMs are able to engage in ePA transactions, and the PBM industry has been at the forefront of efforts to increase the use of ePA to streamline the PA process. Electronic PA not only streamlines the process, thereby reducing administrative burden and reducing fraud, waste, and abuse, and lowers costs, but it is also more secure and expedites the process so that patients can receive needed medications with minimal delay.[4] Despite the many benefits of ePA and regulatory actions to promote and even mandate it, most health care providers still use nonelectronic methods to submit PAs. According to a 2021 AMA survey, physicians reported that most PAs are still done through phone calls and faxes, with only 26 percent reporting that they have an EHR system that supports ePA for prescription medications.[5] In addition, even though many EHR software systems have incorporated ePA capabilities, these functionalities are often not part of standard options due to technical issues, such as non-integration with older legacy systems.

We recommend that CMS consider ways to increase the use of ePA by prescribers. This should include a combination of reasonable mandates to modernize and complementary incentives to help prescribers meet the mandates. Some options include:

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[2] See CAQH. (2020) CAQH Study: Initial Results Show Pharmacy Administrative Transactions Are Highly Automated | CAQH.

[3] CAQH. (2020) *The 2019 CAQH Pharmacy Services Index*. https://www.caqh.org/hubfs/43908627/drupal/explorations/index/index-pharmacy-brief.pdf.

[4] See 89 Fed. Reg. at 8862 (February 8, 2024): "The benefits of using an ePA process improve the manual and burdensome process used today. Making the prior authorization process electronic will reduce the time and burden associated with the current process, allowing providers to put time back into direct patient care, and ultimately will reduce provider burnout."

[5] 87 FR at 76286 (December 13, 2022).

- **Expand the Medicare Part D plan requirement for prescribers to use ePA to all Part D drugs.** Currently, prescribers are only required to use ePA for Part D controlled substances, and it is optional for them to use it for other Part D drugs. We believe CMS has the authority to expand this to require ePA for all Part D drugs pursuant to its authority to require an electronic prescribing program under the Medicare Part D program.

- **Provide meaningful incentives in Medicare Part B for prescribers to engage in ePA.** By providing meaningful financial incentives to prescribers, CMS will provide the assurance that any amount spent by the prescriber on the necessary technical infrastructure and/or EHR system or other functionality will be an investment, rather than an expenditure. While CMS has added an ePA performance measure for the Merit-based Incentive Payment System (MIPS) for eligible health professions and hospitals, this measure does not include PA for drugs.[6] In addition, the other existing performance measures related to the use of certified electronic health record (CERH) technology (CEHRT) for drugs is limited to electronic prescribing transactions and does not include ePA. CMS should, at a minimum, add a performance measure in the MIPS and Promoting Interoperability Programs for the use of ePA for drugs.

- **CMS should develop innovative payer and provider models to demonstrate the savings and efficiency of ePA**. For example, CMMI could propose a model to change payment levels for prescribers and capitated payments to plans based on the level of ePA in which they engage. This would complement the mandate under Part D to use ePA for controlled substances if implemented by CMS, pursuant to its broad program authority, for other Part D drugs.

- **CMS should consider providing funding to smaller and rural providers to obtain EHR systems capable of supporting ePA.** This funding could be in the form of grants or loans (depending on the statutory authority relied upon). Given that only 23% of small and rural providers report that their EHR system supports ePA, this funding is likely to make a measurable difference to the percentage of providers who engage in ePA.

- **HHS should expand the Trusted Exchange Framework and Common Agreement (TEFCA) to support ePA transactions**. CMS should work with ASTP/ONC to expand the voluntary TEFCA to include transactions with payers,

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[6] CMS added an ePA measure to the Health Information Exchange (HIE) objective for the Merit-based Incentive Payment System (MIPS) Promoting Interoperability performance category and the Medicare Promoting Interoperability Program in February 2024.[6] Under this measure, beginning in the 2027 performance period, the 2029 payment year for eligible clinicians and the 2027 EHR reporting period for eligible hospitals and critical access hospitals, the clinician or hospital must attest to having requested a prior authorization electronically via a Prior Authorization API using data from certified electronic health record technology (CEHRT) for at least one hospital discharge (for hospitals) and one nondrug medical item or service (for both hospitals and clinicians).

including ePA. This will not only benefit providers by providing another avenue for the transmission of ePA, but by increasing the utility of TEFCA to payers, it will make it more useful across the healthcare system, improving interoperability and health data exchange more broadly.

- **CMS should revise the medical loss ratio (MLR) requirements to incentivize health plan expenditures to support ePA technology.** The MLR requirements that apply to Medicare, Medicaid, and commercial health plans currently allow health IT costs to be included in the numerator of the ratio if the costs are for activities that improve health care quality or the quality of care, but concurrent and retrospective utilization review activities are explicitly excluded from this category. The regulations should be revised to expand the category of expenses that count toward the numerator of the MLR to explicitly include health IT invested in improving ePA. By doing so, the MLR requirements will change from acting as a disincentive to such expenditures to actually encouraging them.

2. Electronic Prescribing (eRx) and Real Time Benefits Tools (RTBT)

Similar to ePA, eRx and RTBT offer multiple benefits to patients and providers. ERx reduces prescription errors, streamlines workflow, and helps prevent adverse events from medication interactions, leading to cost savings and enhanced patient care. Prescriber-facing RTBT provides full and real-time access to the drugs covered under a patient's prescription benefit plan and the applicable cost sharing, allowing prescribers to choose the most clinically appropriate and cost-effective medication for the patient, eliminating delays in access and lowering costs for the patient. CMS has made progress in driving adoption of both ERx and prescriber RTBT, but more work needs to be done.[7] Both tools are critical to value-based care (VBC) arrangements in that they give providers timely access to essential data to make medication choices that provide the greatest value to patients from a clinical and cost perspective.

While eRx measures are included in the CMS Medicare Promoting Interoperability Program,[8] given the ubiquity of eRx capabilities coupled with the fact that 92% of prescribers already engage in eRx, it is appropriate now to proceed to mandating eRx by all prescribers with perhaps a few very limited exceptions,[9] and to increase incentives to use RTBT. The greater use of RTBT has been shown to significant

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[7] In a May 2019 final rule, CMS required Part D plan sponsors to implement, by January 1, 2021, one or more electronic RTBT capable of integrating with at least one prescriber's e-prescribing system or EHR to provide prescribers with complete, accurate, timely, clinically appropriate, patient-specific formulary and benefit information. In a June 2024 final rule, CMS required that beginning January 1, 2027, Part D sponsors' prescriber RTBT must comply with the standard adopted by ASTP/ONC in 45 CFR 170.205(c), which is the NCPDP RTPB standard version 13.

[8] See https://www.cms.gov/medicare/regulations-guidance/promoting-interoperability-programs/calendar-year-2024-program-requirements.

[9] See A Decade of Data Examined: The Evolution of Electronic Prescribing - Health IT Buzz ("Now, most prescribers have e-prescribing capability integrated in EHRs. Virtually all pharmacies can accept e-prescriptions and 92% of prescribers e-prescribe, an 85 percentage-point increase from 2008 when only 7% of prescribers were sending e-prescriptions").

improvements in prescription fill rates and decreased patient copay costs, despite the fact that prescribers still choose the higher cost medication in most cases when a lower cost alternative is available.[10] Combining incentives to use RTBT with VBC arrangements will likely increase prescriber acceptance of lower cost alternatives, increase medication adherence, and improve health outcomes.

Finally, in addition to increasing the use of pharmacy benefit tools, we recommend that CMS and ASTP/ONC encourage interoperability by focusing on establishing technical standards and resources for all stakeholders to facilitate adoption and uptake of other electronic tools that improve health data exchange, including speed, accuracy, quality, adherence, and overall system satisfaction. This, combined with taking visible and public measures to ensure compliance with the information blocking rule, will increase access to health information, leading to improved health outcomes and lower health care costs.

Conclusion

Thank you for the opportunity to provide comments in response to the RFI. We look forward to working with CMS and ASTP/ONC as they develop policies to promote the seamless and secure flow of health information between patients, caregivers, providers, and payers across the health care spectrum.

Sincerely,

Tim Dube

Tim Dube

Senior Vice President, Policy and Regulatory Insights, PCMA

Cc: Emilia Clements, Associate Vice President, Federal Regulatory Affairs, PCMA
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[10] See https://pubmed.ncbi.nlm.nih.gov/38713809/ ("[Real time prescription benefit] requests received a response for 88% of prescriptions, with price estimates provided for 77.9% of them. Lower-cost alternatives accounted for 67.2% of alerts, while prior authorization requirements represented 15% of alerts. Prescribers selected a lower-cost alternative 32% of the time. For those with an RTPB alert, patients filled prescriptions 68% of the time when an alternative was chosen, compared to 59% of the time when the original prescription was retained").